

言语治疗学 考前复习资料

Speech-Language Therapy Exam Review Guide

★ **Model Answer:** Language is an agreed-upon symbolic system used by a social community to convey meaning, consisting of phonology, vocabulary, and grammar. Speech is the neuromuscular act of converting language into sound. Disorders of language (e.g., aphasia, LDD) differ from disorders of speech (e.g., dysarthria, stuttering).

一、考试信息汇总 Exam Information

| 试卷共两套（A/B 卷），学校随机抽取。

题型 Type	题量 Qty	分值/题 Pts	总分 Total
选择题 MCQ	20	2	40
名词解释 Term Explanation	5	4	20
简答题 Short Answer	5	4	20
论述题 Essay	4	5	20

老师重点提示

- 试卷有两套（A/B 卷），学校随机抽取。两套卷共同重点：失语症 + 吞咽障碍占比最大，嗓音次之。
- 失语症 8 大类型必须背熟，重点在分型鉴别（四维度：流利性/听理解/复述/命名）+ 3 步判别法。
- 失语症治疗：Schuell 刺激法、PACE（促进实用交流能力）、语言治疗环境要求。
- 吞咽障碍：临床检查/评估方法（洼田饮水试验、RSST、VFSS 金标准、FEES、V-VST），治疗方法（气道保护四手法、进食姿势四调整、食物质地分级）。
- 名词解释重点：各种障碍概念——失语症、吞咽障碍、构音障碍、嗓音障碍、听力障碍、语言 vs 言语区别。
- 嗓音障碍：GRBAS 量表（五项含义+0-3 评分），各类型嗓音障碍的临床表现。
- 构音障碍：舌功能与受累音的关系（舌尖音 d/t、舌根音 g/k/h、唇音 b/p/m），构音障碍各类型特征。

二、名词解释 押题（10 候选） Term Explanations

以下覆盖老师提及的所有障碍概念。英文考试需能默写英文定义。每题 4 分，共 20 分（5 选 5）。

1. 失语症 Aphasia ★★★

Key Points:

定义：语言习得后 after language acquisition，因大脑功能受损 due to brain function impairment 导致语言能力丧失或受损 loss/impairment of language abilities，表现为听说读写障碍

常见病因：脑血管病 cerebrovascular disease、颅脑外伤 TBI、脑肿瘤 brain tumour、感染 infection

英文：Aphasia is the loss/impairment of language abilities caused by brain damage after language acquisition, manifesting in deficits of auditory comprehension, oral expression, reading, and writing

★ **Model Answer:** Aphasia is the loss or impairment of language abilities caused by acquired brain damage (e.g., stroke, TBI, tumour) after language has been fully acquired. It affects one or more of the four modalities: auditory comprehension, oral expression, reading, and writing, and is not due to motor speech or sensory deficits alone.

2. 吞咽障碍 Dysphagia ★★★

Key Points:

定义：食物由口腔到胃过程中的困难 difficulty in food passing from mouth to stomach；或因口腔咽部肌肉控制/协调障碍不能正常吞咽

分期：口腔准备期 oral preparatory → 口腔期 oral → 咽期 pharyngeal → 食管期 esophageal

英文：Dysphagia is difficulty in food passage from mouth to stomach, caused by poor control/incoordination of oral & pharyngeal muscles

★ **Model Answer:** Dysphagia is difficulty in the passage of food or liquid from the mouth to the stomach, caused by impaired neuromuscular control or structural abnormalities of the oral, pharyngeal, or oesophageal stages of swallowing. Its most serious complication is aspiration pneumonia.

3. 语言 vs 言语 Language vs Speech ★★★

Key Points:

语言 Language：人类社会约定俗成的符号系统 an agreed-upon symbolic system；以语音为载体、语义为内容的词汇与语法规则体系。代表障碍：失语症 aphasia、语言发育迟缓

言语 Speech: 表达语言思维的方式 the way to express linguistic thinking; 构成有声语言的机械过程 the mechanical process of forming spoken language, 需神经肌肉参与。代表障碍: 构音障碍 dysarthria

一句话: 语言=符号系统; 言语=把它说出来的机械过程

4. 构音障碍 Dysarthria ★★

Key Points:

定义: 因构音器官先天/后天结构异常或神经/肌肉功能障碍所致的构音异常 articulation disorder

与失语症区别: 构音障碍是"说不好"(运动执行); 失语症是"不会说"(语言编码)

英文: Dysarthria is an articulation disorder caused by structural abnormalities of articulatory organs or neurological/muscular dysfunction

★ **Model Answer:** Dysarthria is a motor speech disorder resulting from neurological damage or structural abnormalities of the articulatory organs, causing weakness, paralysis, or incoordination of the speech musculature. Unlike aphasia, language formulation is intact — the deficit lies in execution.

5. 嗓音障碍 Voice Disorder ★★

Key Points:

定义: 因呼吸及喉调节机制器质性或功能性异常引起; 表现: 失声 aphonia、发声困难、声嘶 hoarseness

发声三要素: 动力源 power source (肺)、振动源 vibration source (声带)、共鸣腔 resonance cavities

英文: Voice disorder is caused by organic/functional abnormalities of respiratory & laryngeal mechanisms, manifesting as aphonia, hoarseness, or phonation difficulty

★ **Model Answer:** A voice disorder is an abnormality of pitch, loudness, or voice quality caused by organic or functional disorders of the respiratory and laryngeal systems. It may manifest as hoarseness, aphonia, or effortful phonation, and is classified as organic or functional depending on aetiology.

6. 听力障碍 Hearing Impairment ★★

Key Points:

定义：传音/感音/中枢听觉处理器质性或功能性异常，致不同程度听力减退；轻度=重听 hard of hearing，重度=聋 deafness

分类：传导性 conductive / 感音神经性 sensorineural / 混合性 mixed / 中枢性 CAPD

英文：Hearing impairment is organic or functional abnormality of sound conduction, perception, or central auditory processing

★ **Model Answer:** Hearing impairment is a reduction in hearing sensitivity caused by conductive, sensorineural, mixed, or central auditory processing disorders. Severity is graded by pure-tone average: mild (26–40 dB), moderate (41–55 dB), moderately severe (56–70 dB), severe (71–90 dB), and profound (≥ 91 dB).

7. 口吃 Stuttering ★

Key Points:

定义：言语流畅性障碍 fluency disorder，表现为字/音重复 repetition、停顿 pauses、声音延长 prolongation

英文：Stuttering is a fluency disorder characterized by sound/syllable repetition, pauses, and prolongation

★ **Model Answer:** Stuttering is a fluency disorder characterised by involuntary repetitions, prolongations, and blocks of sounds, syllables, or words, often accompanied by secondary behaviours (e.g., avoidance, tension). Onset is typically in early childhood; exact aetiology is multifactorial.

8. 误吸 Aspiration ★★

Key Points:

定义：食物/液体/分泌物进入声门以下气道 entry of material below the true vocal cords into the trachea

隐性误吸 silent aspiration：无明显呛咳的误吸，最危险；并发症：吸入性肺炎 aspiration pneumonia（可致死）

英文：Aspiration is the entry of material below the vocal cords into the trachea, which can lead to aspiration pneumonia

★ **Model Answer:** Aspiration is the entry of material (food, liquid, secretions, or refluxate) below the true vocal cords into the trachea and lungs. Silent aspiration occurs without any cough reflex and is particularly dangerous, as it can lead to aspiration pneumonia, the most life-threatening complication of dysphagia.

9. VFSS 电视透视吞咽造影 ★

Key Points:

定义: Videofluoroscopic Swallow Study, 吞咽障碍评估的影像学金标准 the imaging gold standard for dysphagia assessment

可观察口腔期、咽期、食管期完整吞咽过程, 评估误吸和残留

英文: VFSS is the gold-standard imaging method for evaluating all swallowing stages and detecting aspiration

★ **Model Answer:** Videofluoroscopic Swallow Study (VFSS) is the gold-standard instrumental assessment for dysphagia. Using real-time fluoroscopy with radio-opaque contrast medium, it visualises all four swallowing stages and directly detects aspiration, penetration, and residue, enabling targeted intervention planning.

10. PACE 促进实用交流能力 ★★

Key Points:

定义: Promoting Aphasics' Communicative Effectiveness, 失语症治疗方法

四项原则: 新信息交换、自由选择交流手段、医患平等参与、按信息传递效果反馈

英文: PACE is an aphasia treatment emphasizing new information exchange, free choice of communication means, equal participation, and feedback based on transmission success

★ **Model Answer:** PACE (Promoting Aphasics' Communicative Effectiveness) is an aphasia treatment approach based on four principles: exchange of new information, free choice of communication channel (speech, gesture, drawing, AAC), equal participation between clinician and patient, and feedback based on communicative success rather than linguistic accuracy.

三、简答题 押题 (10 候选) Short Answer Questions

每题 4 分，共 20 分（5 选 5）。答出要点即可，不必长篇展开。

1. 失语症 8 大类型及四维度鉴别 Briefly describe the 8 types of aphasia and 4 classification axes

Key Points:

四维度：流利性 Fluency、听理解 Comprehension、复述 Repetition、命名 Naming

8 大类型逐型写出：Broca(非流利/听理解好/复述差)、Wernicke(流利/听解差/复述差)、传导性(流利/听解好/复述差突出)、经皮质运动性(非流利/听解好/复述好)、经皮质感觉性(流利/听解差/复述好)、经皮质混合性(非流利/听解差/复述好)、完全性(四项全差)、命名性(仅命名差)

3 步判别：①复述→分经皮质类/外侧裂周类；②流利性→前/后部；③听理解→锁定类型

2. 吞咽障碍的临床检查/评估方法 Clinical assessment methods for dysphagia

Key Points:

洼田饮水试验 Water Swallow Test: 30ml 温水，5 级，I 级正常，≥III 级提示障碍

RSST 反复唾液吞咽试验：30 秒空吞咽≥3 次正常，<3 次异常（不能查隐性误吸）

VFSS = 金标准 gold standard；FEES = 内镜直视咽喉

V-VST：评估安全性 safety + 有效性 efficacy

3. 气道保护四手法 4 Airway-Protection Maneuvers

Key Points:

声门上吞咽 Supraglottic swallow: 吞咽前后屏气闭声门→再咳清残留

超声门上吞咽 Super-supraglottic swallow: 更用力屏气 forceful breath-hold

用力吞咽 Effortful swallow: 增强舌根后移 tongue-base retraction

Mendelsohn 手法：延长喉上抬 prolong laryngeal elevation，保持 2-5 秒促环咽开放

+Masako 舌制吞咽 tongue-hold swallow: 增强舌根力量

4. GRBAS 嗓音评估量表 GRBAS voice assessment scale

Key Points:

主观听感评估，4 级评分 0-3（0=正常，3=重度）

G=Grade 总嘶哑度；R=Roughness 粗糙声；B=Breathiness 气息声；A=Asthenia 无力声；
S=Strain 紧张声

5. Schuell 刺激法 Schuell stimulation approach

Key Points:

核心：适当的听觉刺激 appropriate auditory stimulation

原则：多途径 multi-modal；反复刺激并引出反应 repeated stimulation eliciting responses

环境：安静/一对一/30min 每日 1 次/3 月疗程/上午为宜

6. PACE 四项原则 4 Principles of PACE

Key Points:

新信息交换 new information exchange

自由选择交流手段 free choice of communication means

医患平等参与 equal participation

按信息传递效果反馈 feedback based on message transmission success

7. 语言治疗环境要求 Therapy environment requirements

Key Points:

物理环境：安静 quiet、隔音吸音 sound-insulated/absorbent、避免视听干扰、房间简洁

形式：一对一为主 one-to-one

节奏：30min/次、每日 1 次 30 min/session, once daily、3 月一疗程 3-month course；上午为宜 mornings preferred

8. 吞咽障碍四分期 4 Stages of swallowing

Key Points:

口腔准备期 Oral preparatory: 咀嚼搅拌→食团形成 bolus formation

口腔期 Oral: 舌将食团推向咽部 tongue propels bolus

咽期 Pharyngeal: 吞咽反射触发→喉上抬/声门关闭/环咽开放

食管期 Esophageal: 蠕动入胃 peristalsis to stomach

9. 舌功能与受累音 Tongue function & affected sounds

Key Points:

舌尖音 tongue-tip: d、t (舌尖抵上齿龈)

舌根音 tongue-back/dorsum: g、k、h (舌根与软腭)

唇音 labial: b、p、m (唇闭合)

10. 进食姿势四调整 4 Posture Adjustments

Key Points:

低头吞咽 Chin-down: 易误吸者 aspiration-prone, 保护气道

仰头吞咽 Chin-up: 舌运动差 poor tongue mobility

转头吞咽 Head rotation: 向患侧 to affected side, 清梨状窝残留

侧头吞咽 Head tilt: 向健侧 to healthy side, 借健侧推送

四、论述题 押题 (8 候选) Essay Questions

每题 5 分, 共 20 分 (4 选 4)。需展开完整框架+关键细节。失语症和吞咽障碍是核心。

1. 失语症 8 大类型分型鉴别与判别流程 Classification & differentiation of 8 aphasia types

★★★

Key Points:

引言: 四维度——流利性 Fluency、听理解 Comprehension、复述 Repetition、命名 Naming

逐型展开 (每型写出四维度+特征+病灶):

Broca: 非流利/听理解好/复述差/命名差——"能听懂说不出", 左额下回后部

Wernicke: 流利/听理解差/复述差/命名差——"说得多听不懂", 左颞上回后部

传导性 **Conduction:** 流利/听解好/复述差突出/命名差——弓状束 arcuate fasciculus

经皮质运动性 **TCM:** 非流利/听解好/复述好/命名差——"像 Broca, 复述好"

经皮质感觉性 **TCS:** 流利/听解差/复述好/命名差——"像 Wernicke, 复述好"

经皮质混合性 **MT:** 非流利/听解差/复述好/命名差——"几乎全坏, 独留复述"

完全性 **Global:** 四项全差——最重 most severe

命名性 **Anomic:** 仅命名差——"只差命名"

3 步判别法: ①复述好/差分经皮质/外侧裂周; ②流利/非流利分前/后部; ③听理解锁定类型

总结: 复述是第一把钥匙 first key

2. 吞咽障碍评估与治疗综合 **Assessment & treatment of dysphagia ★★★**

Key Points:

评估 **Assessment:**

临床: 洼田饮水试验 (30ml/5 级/I 级正常)、RSST (30 秒≥3 次正常)

仪器: VFSS (金标准)、FEES (内镜直视)、V-VST (安全+有效)

治疗 **Treatment:**

气道保护四手法: 声门上/超声门上/用力吞咽/Mendelsohn + Masako

进食姿势四调整: 低头(易误吸)/仰头(舌差)/转头(向患侧)/侧头(向健侧)

食物质地分级: 稀流质→浓流质→糊状→半固体→固体; 易吞咽食物特点: 密度均匀、适当粘性、不易松散、残留少

其他: 口腔运动训练 oral motor training、冷刺激 cold stimulation(吞咽失用/启动延迟)、球囊扩张 balloon dilation(环咽功能障碍)

总结: 评估先行→治疗个体化 (气道+姿势+饮食综合管理)

3. 失语症主要治疗方法及原理 **Treatment methods for aphasia ★★**

Key Points:

Schuell 刺激法: 适当听觉刺激→多途径→反复刺激引出反应

PACE: 新信息交换/自由选择交流手段/医患平等/按效果反馈

阻断去除法 Deblocking: 去除对残存功能的抑制

程序学习法 Programmed learning: 分步渐进

MIT 旋律语调治疗: 利用右脑旋律补偿左脑语言损伤

小组治疗 Group therapy / AAC 辅助替代交流

语言治疗环境: 安静/隔音/一对一/30min 每日 1 次/3 月疗程/上午

4. GRBAS 评估 + 嗓音障碍各类型表现 GRBAS & clinical manifestations of voice disorders ★★

Key Points:

GRBAS: 0-3 四级评分, G 总嘶哑度/R 粗糙声/B 气息声/A 无力声/S 紧张声

声带小结 Vocal nodules: 用声过度→持续声嘶

声带息肉 Vocal polyp: 声嘶-费力-粗糙 hoarse-effortful-rough

慢性喉炎 Chronic laryngitis: 说久声嘶+粘痰

声带水肿 Vocal-cord edema: 吸烟/粉尘→发声疲劳

痉挛性发音障碍 Spasmodic dysphonia: 喉肌紧张, 声音中断 voice breaks

心因性失声 Psychogenic aphonia: 情绪诱发, 只动嘴无声, 咳/笑正常

最常见病因: 用声过度 vocal overuse/abuse

5. 吞咽障碍并发症及预防管理 Complications & prevention ★★

Key Points:

并发症: 误吸 aspiration→吸入性肺炎 aspiration pneumonia (最严重可致死); 隐性误吸 silent aspiration (最危险); 营养不良 malnutrition; 脱水 dehydration

预防: 评估先行 VFSS/FEES; 气道保护手法; 姿势调整 (低头吞咽); 食物质地调整; 口腔卫生 oral hygiene; 严重者管饲 tube feeding

6. 构音障碍各类型主要言语特征 Classification & speech characteristics of dysarthria ★★

Key Points:

弛缓型 Flaccid: 下运动神经元→气息声 breathy voice、鼻音过重 hypernasality

痉挛型 Spastic: 上运动神经元→费力言语 effortful speech、粗糙声（肌张力高 hypertonia）

运动失调型 Ataxic: 小脑→不规则构音、韵律异常

运动低下型 Hypokinetic: 锥体外系(帕金森)→单响度 monoloudness、单音调 monotone、短促

运动过多型 Hyperkinetic: 锥体外系(亨廷顿)→不自主运动影响构音

混合型 Mixed: 多种组合(如 ALS)

7. 听力障碍分类分级及对言语发展影响 Classification & grading of hearing impairment ★

Key Points:

按病变部位: 传导性 conductive / 感音神经性 sensorineural / 混合性 mixed / 中枢性 CAPD

分级: 轻度 26-40/中度 41-55/中重度 56-70/重度 71-90/极重度≥91 dB

对言语影响: 听力是口语发展前提 auditory input = prerequisite for oral language; 重度聋→言语严重受限; 早期干预抓住关键期 critical period

8. S-S 评估法及语言发育迟缓早期干预原则 S-S method & early intervention ★

Key Points:

S-S 法适用年龄上限约 6.5 岁, 三方面评估: ①符号形式-指示内容关系 (阶段 1-4); ②基础过程 (听觉记忆/视觉认知/模仿); ③交流态度 (眼神/轮流/主动性)

阶段 3 (符号阶段) 区分: 手势符号 visual-motor circuit vs 语音符号 auditory-speech circuit

早期干预原则: 抓住关键期 critical period; 基于评估的个体化方案 individualized program; 一对一为主 one-to-one; 重视交流态度; 家庭参与

五、复习笔记正文 Review Notes（完整版）

以下为完整复习笔记，涵盖总论、失语症、吞咽障碍、嗓音障碍、构音障碍、听力障碍、量表总结、英文缩写、考试重点提示。配合前面押题部分使用效果最佳。

言语治疗学复习笔记 Speech-Language Therapy Review Notes

卷面 Paper: 选择 MCQ $20 \times 2 = 40$ 名解 Term explanation $5 \times 4 = 20$ 简答 Short answer $5 \times 4 = 20$
论述 Essay $4 \times 5 = 20$ 重点 Priority: 失语症 Aphasia + 吞咽障碍 Dysphagia 占大头; 嗓音 Voice 次之; 构音 Dysarthria、听力 Hearing 主要出名解/选择。

一、总论与名词解释 Overview & Key Definitions

名解 $5 \times 4 = 20$ 分，先背死。英文考试要能默写英文。

语言与言语的区别 Language vs Speech（老师点名）：

- **语言 Language:** 人类社会约定俗成的符号系统 an agreed-upon symbolic system in human society; 以语音为载体、语义为内容的词汇与语法规则体系 a system of vocabulary (phonetics as carrier, semantics as content) and grammatical rules。代表性障碍 Representative disorders: 失语症 aphasia、语言发育迟缓 language developmental delay。
- **言语 Speech:** 表达语言思维的方式 the way to express linguistic thinking; 构成有声语言的机械过程 the mechanical process of forming spoken language, 需神经肌肉参与发声器官运动 involving vocal organs with nerves and muscles。代表性障碍 Representative disorder: 构音障碍 dysarthria。
- **一句话 Key:** 语言=符号系统 the symbol system; 言语=把它说出来的机械过程 the mechanical act of producing it。

各障碍定义 Definitions of Disorders:

- **失语症 Aphasia:** 语言习得后 after language acquisition, 因大脑功能受损 impairment of brain function 导致语言能力丧失或受损 loss/impairment of language abilities, 表现为听说读写障碍。常见病因 Causes: 脑血管病 cerebrovascular disease、颅脑外伤 TBI、脑肿瘤 brain tumour、感染 infection。
- **构音障碍 Dysarthria:** 因构音器官先天/后天结构异常 structural abnormalities of articulatory organs 或神经/肌肉功能障碍 neurological/muscular dysfunction 所致的构音异常 articulation disorder。

- **嗓音障碍 Voice disorder / 发声障碍 phonation disorder:** 多由呼吸及喉调节机制器质性或功能性异常 organic/functional abnormalities of respiratory & laryngeal mechanisms 引起；表现 manifestations: 失声 aphonia、发声困难、声嘶 hoarseness。
- **吞咽障碍 Dysphagia:** 食物由口腔到胃过程中的困难 difficulty in food passing from mouth to stomach, 或因口腔咽部肌肉控制/协调障碍 poor control/incoordination of oral & pharyngeal muscles 而不能正常吞咽。
- **听力障碍 Hearing impairment:** 传音 sound conduction、感音 sound perception 或中枢听觉综合分析系统 central auditory processing 发生器质性/功能性异常，致不同程度听力减退。轻度=重听 hard of hearing, 重度=聋 deafness。
- **口吃 Stuttering:** 言语流畅性障碍 a fluency disorder of speech, 表现为字/音重复 repetition、停顿 pauses、声音延长 prolongation。

二、失语症 Aphasia (头号重点 ★★★)

你的PPT无此章，按李胜利第3版补。重点=分型与鉴别。

临床表现 Clinical Manifestations: 听理解 auditory comprehension、口语表达 oral expression、阅读 reading、书写 writing 四方面障碍（如命名障碍 anomia、复述障碍 repetition impairment、错语 paraphasia、刻板言语 stereotyped speech、找词困难 word-finding difficulty）。

分型四维度 4 Axes for Classification: 自发言语流利性 Fluency、听理解 Comprehension、复述 Repetition、命名 Naming。

类型 Type	流利性 Fluency	听理解 Comprehension	复述 Repetition	命名 Naming	特征 Key cue
Broca (运动性 motor)	非流利 non-fluent	较好 good	差 poor	差 poor	能听懂、说不出
Wernicke (感觉性 sensory)	流利 fluent	差 poor	差 poor	差 poor	说得多、听不懂
传导性 Conduction	流利 fluent	好 good	**差 poor (突出) **	差 poor	听懂能说，复述坏
经皮质运动性 Transcortical motor	非流利 non-fluent	好 good	**好 spared**	差 poor	像 Broca, 复述好
经皮质感觉性 Transcortical sensory	流利 fluent	差 poor	**好 spared**	差 poor	像 Wernicke, 复述好
经皮质混合性 Mixed transcortical	非流利 non-fluent	差 poor	**好 spared**	差 poor	几乎全坏，独留复述
完全性 Global	非流利 non-fluent	差 poor	差 poor	差 poor	四项全差，最重
命名性 Anomic	流利 fluent	好 good	好 good	**差 poor (突出) **	只差命名

3 步判别 Quick Rule:

失语症 8 型鉴别对照表 8-Type Aphasia Quick Reference Table

Type 类型	Fluency 流利性	Comprehension 听理解	Repetition 复述	Naming 命名	Classic Lesion
Broca	X Non-fluent	✓ Good	X Poor	X Poor	L. inferior frontal gyrus
Wernicke	✓ Fluent	X Poor	X Poor	X Poor	L. superior temporal gyrus
Conduction 传导性	✓ Fluent	✓ Good	X X Very poor	X Poor	Arcuate fasciculus
Global 完全性	X Non-fluent	X Poor	X Poor	X Poor	Large perisylvian
Anomic 命名性	✓ Fluent	✓ Good	✓ Good	X Poor only	Variable
TC Motor 经皮质运动	X Non-fluent	✓ Good	✓ Good	X Poor	Anterior border zone
TC Sensory 经皮质感觉	✓ Fluent	X Poor	✓ Good	X Poor	Posterior border zone
TC Mixed 经皮质混合	X Non-fluent	X Poor	✓ Good	X Poor	Bilateral border zone

3-step rule: ① Repetition → transcortical (TC) vs perisylvian; ② Fluency → anterior vs posterior; ③ Comprehension → locks type.

- ① 复述好 repetition spared → 经皮质类 transcortical; 复述差 → 外侧裂周类 perisylvian (Broca/Wernicke/传导) + 完全性。
- ② 流利 fluent → 后部病灶 posterior (Wernicke 类); 非流利 non-fluent → 前部 anterior (Broca 类)。
- ③ 听理解差 poor comprehension → Wernicke / 经皮质感觉 / 混合。
- ④ 仅命名差 = 命名性失语 Anomic。

治疗方法 Treatment Methods:

- **Schuell 刺激法 Stimulation approach:** 适当的听觉刺激 appropriate auditory stimulation、多途径 multi-modal、反复刺激并引出反应 repeated stimulation eliciting responses。
- **PACE (促进实用交流能力 Promoting Aphasics' Communicative Effectiveness):** 新信息交换 new information exchange、自由选择交流手段 free choice of means、医患平等参与 equal participation、按信息传递效果反馈 feedback by message transmission。
- **认名字即可 Recognise only:** 阻断去除法 deblocking、程序学习法 programmed learning、旋律语调治疗 MIT、小组治疗 group therapy、交流板/替代交流 AAC。

语言治疗环境 Therapy Environment (简答可用): 安静 quiet、隔音吸音 sound-insulated/absorbent、避免视听干扰 free of visual/auditory distraction、房间简洁; 以一对一

为主 one-to-one; 每次约 30 分钟、每日 1 次、疗程 3 个月 30 min/session, once daily, 3-month course; 上午为宜 mornings preferred。

三、吞咽障碍 Dysphagia (重点 ★★★)

分期 Stages (5 期 / clinically simplified to 4) :

- ① 口腔前期 **Pre-oral / Anticipatory phase**: Cognitive & sensory preparation BEFORE food enters the mouth. Involves visual and olfactory recognition of food, salivation triggered by anticipation, jaw and lip positioning, and utensil handling. Absent in cognitively impaired patients (e.g., severe dementia).
- ② 口腔准备期 **Oral preparatory phase**: Food is taken into the mouth; chewing and mixing with saliva forms a cohesive bolus. Requires lip seal, jaw movement, tongue lateralisation, and buccal tension.
- ③ 口腔期 **Oral phase**: Tongue tip elevates and propels bolus posteriorly toward pharynx. Duration ~1 s. Voluntary and under cortical control.
- ④ 咽期 **Pharyngeal phase**: Swallowing reflex triggered at faucial pillars. Sequence: velopharyngeal closure (prevent nasal regurgitation) → laryngeal elevation & epiglottic tilt → vocal fold adduction → cricopharyngeal opening. Duration ~1 s. Involuntary.
- ⑤ 食管期 **Oesophageal phase**: Bolus transported by peristalsis to stomach. Duration 8–20 s. Managed by gastroenterology if impaired.

Note: ①②③ are sometimes grouped as 'oral stage' in simplified 4-stage model.

分类 **Classification**: 器质性 organic vs 功能性 (神经源性) functional (neurogenic)。

临床表现/并发症 **Manifestations / Complications**: 呛咳 coughing、误吸 aspiration、鼻腔反流 nasal regurgitation、湿性嗓音 wet voice、咽部残留 pharyngeal residue。并发症 **Complications**: 误吸 aspiration、吸入性肺炎 aspiration pneumonia、营养不良 malnutrition、脱水 dehydration (重者可致死)。

评估 **Assessment** (老师明说"临床检查"是重点) :

- 洼田饮水试验 **Water Swallow Test (Kubota)**: 喝 30ml 温水, 分 5 级; 1 级 (5 秒内一次咽下、无呛咳) = 正常 normal, 3 级及以上 ≥ Grade 3 提示障碍。
- **Water Swallow Test (WST) — Full 5-Level Grading**:

Grade I (Normal): Swallows 30 ml water in one attempt within 5 seconds, no coughing.

Grade II (Borderline): Swallows 30 ml water in one attempt but takes >5 seconds, OR completes in 2 attempts without coughing.

Grade III (Mild impairment): Swallows 30 ml water in one attempt with coughing.

Grade IV (Moderate impairment): Requires 2+ attempts to swallow 30 ml and coughing occurs.

Grade V (Severe impairment): Frequent coughing; unable to swallow 30 ml even with multiple attempts.

Clinical cut-off: Grade I = normal; Grade II = borderline (follow-up); Grade III–V = dysphagia present. Grade III+ → refer for VFSS/FEES.

- **反复唾液吞咽试验 RSST (Repetitive Saliva Swallowing Test):** 30 秒内空吞咽次数 dry swallows in 30 s, ≥3 次正常、<3 次异常（不能查隐性误吸 cannot detect silent aspiration）。
- **SSA (Standardised Swallowing Assessment):** 3-stage bedside screen. Stage 1: consciousness & posture check. Stage 2: teaspoon of water ×3 — observe for cough, wet voice, drooling. Stage 3: 60 ml water — same observations. Abnormal sign at any stage = fail → refer for instrumental assessment. Advantage: simple, no equipment.
- **GUSS (Gugging Swallowing Screen):** 2-part screen for post-stroke dysphagia. Part 1 (indirect): saliva swallow — observe drooling, voluntary cough, voice change (max 5 pts). Part 2 (direct): semi-solid → liquid → solid, 3 pts each. Total 20 pts. ≥20 = minimal risk; 15–19 = mild; 10–14 = moderate; 0–9 = severe. Dietary recommendation attached to each severity level.
- **VFSS (电视透视吞咽造影 Videofluoroscopic Swallow Study) = 金标准 gold standard。**
- **FEES (纤维内镜吞咽检查 Fiberoptic Endoscopic Evaluation of Swallowing) :** 直视咽喉。
- **V-VST (容积-粘度测试 Volume-Viscosity Swallow Test) :** 评估安全性 safety + 有效性 efficacy。

Protocol: 3 viscosities × 3 volumes (9 trials). Viscosities: thin liquid → nectar-thick → pudding-thick. Volumes per level: 5 ml → 10 ml → 20 ml (stop early if safety signs appear).

Safety signs (impaired airway protection): ① Coughing during/after swallow ② Voice change (wet/gurgly quality) ③ O₂ desaturation ≥3%

Efficacy signs (poor bolus transport): ① Labial seal failure ② Oral/pharyngeal residue ③ Multiple swallows needed for one bolus

Clinical use: Identifies the safest and most efficient texture/volume combination for immediate dietary recommendation. Quick bedside tool — no radiation, no equipment.

治疗 Treatment（老师明说"治疗是重点"，下面两组是论述/简答命根子）：

- **气道保护四手法 Airway-Protection Maneuvers:**
 - 声门上吞咽 Supraglottic swallow: 吞咽前后屏气闭声门 hold breath to close glottis, 再咳清残留。
 - 超声门上吞咽 Super-supraglottic swallow: 更用力屏气 forceful breath-hold ("bear down")。
 - 用力吞咽 Effortful swallow: 增强舌根后移 tongue-base retraction, 清咽部残留。
 - Mendelsohn 手法 Mendelsohn maneuver: 延长喉上抬 prolong laryngeal elevation、促环咽开放 open cricopharyngeus, 保持 2–5 秒。
 - (+ Masako 舌制吞咽 tongue-hold swallow: 增强舌根力量)
- **进食姿势四调整 Posture Adjustments:**
 - 低头吞咽 Chin-down / chin-tuck: 易误吸者 aspiration-prone。
 - 仰头吞咽 Chin-up / head-back: 舌运动差 poor tongue mobility。
 - 转头吞咽 Head rotation: 转向患侧 to affected side, 清梨状窝残留。
 - 侧头吞咽 Head tilt: 转向健侧 to healthy side, 借健侧推送。
- 其余 Others: 口腔运动训练 oral motor training (唇/颌/舌 + 腭咽闭合 palatopharyngeal closure、声带闭合 vocal-fold closure); 冷刺激 cold/thermal stimulation (适应症=吞咽失用 swallowing apraxia、感觉减退、启动延迟); 食物质地分级 food-texture grading (稀流质 thin liquid → 浓流质 thickened → 糊状 pureed → 半固体 semi-solid → 固体 solid) + 易吞咽食物特点 (密度均匀、有适当粘性、不易松散、残留少); 球囊扩张 catheter balloon dilation (适应症=环咽肌功能障碍 cricopharyngeal dysfunction)。
- **食物增稠剂 Food thickeners / Dysphagia thickeners:**

Types 类型: ① Starch-based (淀粉基): cheaper, may thin over time with amylase in saliva — less reliable for long-term use. ② Xanthan gum-based (黄原胶基): stable in saliva, does not thin — preferred in clinical practice.

IDDSI Framework (International Dysphagia Diet Standardisation Initiative): 7 levels (0–7). Drinks: Level 0 = thin; Level 1 = slightly thick; Level 2 = nectar-thick (mildly thick); Level 3 = liquidised (moderately thick); Level 4 = puréed (extremely thick). Foods: Level 3 = liquidised; Level 4 = puréed; Level 5 = minced & moist; Level 6 = soft & bite-sized; Level 7 = regular.

Clinical principle: Match texture/thickness to the patient's impaired stage: thin liquid → aspiration risk → thicken; poor chewing → purée. Goal: safest texture that still allows adequate oral intake and nutrition.

四、嗓音障碍 Voice Disorder (中等★★)

发声三要素（条件） 3 Conditions for Phonation: 动力源（肺/呼吸）power source (lungs)、振动源（声带）vibration source (vocal cords)、共鸣腔 resonance cavities。（单选：振动产生声音=声带）

嗓音三要素 3 Voice Characteristics: 音调（频率）pitch (frequency)、响度（强度/振幅）loudness (intensity/amplitude)、音色 timbre。

分类 Classification: 器质性 organic vs 功能性 functional。

GRBAS 量表（老师点名，必背）： 4 级评分 0–3（0 正常、3 重度）。

- **G** 总嘶哑度 Grade (overall hoarseness)
- **R** 粗糙声 Roughness
- **B** 气息声 Breathiness
- **A** 无力声 Asthenia
- **S** 紧张声 Strain

常见类型及表现 Common Types & Signs:

- 声带小结 Vocal nodules: 用声过度 vocal overuse, 持续声嘶。
- 声带息肉 Vocal polyp: 声嘶-费力-粗糙 hoarse-effortful-rough。
- 慢性喉炎 Chronic laryngitis: 说久声嘶 + 粘痰。
- 声带水肿 Vocal-cord edema: 吸烟/粉尘, 发声疲劳。
- 痉挛性发音障碍 Spasmodic dysphonia: 喉肌紧张、说话时声音中断 voice breaks。
- 心因性失声 Psychogenic aphonia: 情绪诱发, 只动嘴无声, 咳/笑正常。

最常见病因 Most Common Cause: 用声过度 vocal overuse/abuse。

嗓音障碍治疗 Voice Disorder Treatment

- **Vocal hygiene education 嗓音卫生教育:** Reduce vocal abuse (avoid shouting, whispering, excessive throat-clearing); maintain hydration; avoid irritants (smoking, alcohol, dry air). First-line for almost all cases.
- **Voice therapy / Behavioural training 发声行为训练:** Techniques include: resonant voice therapy, semi-occluded vocal tract exercises (lip trill, straw phonation), easy onset phonation, and pitch/loudness shaping to reduce laryngeal hyperfunction.
- **Push-pull exercises 推撑练习:** Used for unilateral vocal fold paralysis (breathy, weak voice). Patient pushes against a wall or pulls chair arms while phonating — increases adductory force and mediates the paralysed fold.
- **Laryngeal massage & relaxation 喉部放松:** For hyperfunctional voice disorders (spasmodic dysphonia, muscle tension dysphonia) — manual circumlaryngeal massage reduces extrinsic laryngeal muscle tension.

- **Psychogenic aphonia** 心因性失声 — **psychological intervention**: Voice is present during reflexive acts (cough, laugh) but absent during speech. Treatment: confidential voice technique, counselling, and immediate trial of normal voice production.
 - **Surgical / Medical** 手术/药物: Vocal nodules/polyps → microsurgery after failed conservative treatment; laryngeal papilloma → surgical removal; spasmodic dysphonia → botulinum toxin (Botox) injection into thyroarytenoid muscle.
-

五、构音障碍 **Dysarthria** (扫尾 ★)

定义 **Definition**: 见名解 see definitions above.

舌功能与受累的声音 **Tongue Function & Affected Sounds** (老师举的例子):

- 舌尖音 tongue-tip sounds: d、t (舌尖抵上齿)。
 - 舌根音 tongue-back/dorsum sounds: g、k、h。
 - 唇音 labial sounds: b、p、m (看唇)。
-

六、听力障碍 **Hearing Impairment** (扫尾 ★)

定义 **Definition**: 见名解 see definitions above.

按病变部位分类 **Classification by Lesion Site** (重点): 传导性 **conductive**、感音神经性 **sensorineural**、混合性 **mixed** (+ 中枢听处理障碍 **central auditory processing disorder**, CAPD)。

Differentiating the 4 types — key features:

Conductive 传导性: Lesion: outer or middle ear (e.g., otitis media, wax, otosclerosis). Air conduction (AC) reduced; bone conduction (BC) normal → AB gap ≥ 10 dB. Low frequencies often more affected. Weber: lateralises to WORSE ear. Rinne: BC > AC (negative Rinne). Usually treatable medically/surgically.

Sensorineural 感音神经性: Lesion: cochlea (sensory) or auditory nerve (neural). Both AC and BC reduced equally — no AB gap. High frequencies typically affected first (e.g., noise-induced, presbycusis). Weber: lateralises to BETTER ear. Rinne: AC > BC (positive, but both reduced). Management: hearing aids, cochlear implant.

Mixed 混合性: Both conductive and sensorineural components present. BC reduced (sensorineural component) + additional AB gap (conductive component). Common in chronic otitis media with cochlear involvement.

CAPD 中枢性 (Central Auditory Processing Disorder): Normal peripheral hearing thresholds but difficulty processing auditory information centrally. Difficulty understanding speech in noise, following rapid speech, or with competing signals. Assessment: specialised central auditory tests (dichotic listening, temporal processing). Common in children with learning disabilities.

Common conditions to know: Otitis media 中耳炎 (conductive); Presbycusis 老年性聋 (high-freq sensorineural); Noise-induced 噪声性聋 (4000 Hz notch, sensorineural); Congenital hearing loss 先天性聋 (genetic or prenatal); Acoustic neuroma 听神经瘤 (unilateral sensorineural + tinnitus).

耳聋分级 Grading of Hearing Loss: 轻度 mild 26–40dB、中度 moderate 41–55、中重度 moderately severe 56–70、重度 severe 71–90、极重度 profound ≥91dB。

其余（耳的解剖、测听方法、助听器/人工耳蜗、耳聋预防）老师只要概念，基本不用碰。

七 a. 语言发育迟缓 / S-S 法 Language Developmental Delay & S-S Assessment

- **Definition 定义:** Language developmental delay (LDD) refers to children whose language development significantly lags behind age norms without explanation by hearing loss, intellectual disability, or motor disorder alone. The sign-significate (S-S) method is the gold-standard standardised assessment for children aged approx. 1.5–6.5 years.
- **S-S Method — 3 Assessment Domains:** ① Sign-significate relationship (symbol stages 1–4 / pre-symbolic → symbolic); ② Basic processes (auditory memory, visual recognition, imitation); ③ Communication attitude (eye contact, turn-taking, initiation).
- **Symbol Stages (Stage 3 is the key split):** Stage 1: No response to objects. Stage 2: Object permanence / situational understanding. Stage 3a: Visual-motor (gesture) symbols. Stage 3b: Auditory-speech (verbal) symbols. Stage 4: Words → sentences → grammar.
- **Early Intervention Principles 早期干预原则:** ① Exploit the critical period (语言发展关键期) — intervene as early as possible. ② Individualised programme based on S-S stage. ③ One-to-one sessions; prioritise communication attitude before language targets. ④ Family involvement is essential — parents carry out home practice.
- **Exam note:** If asked 'what does the S-S method assess?' → name all 3 domains and describe Stage 3 split. If asked 'early intervention principles' → list the 4 above with brief rationale.

七、重要量表与评估方法总结 Summary of Key Scales & Assessments

量表/方法 Scale / Method	领域 Domain	要点 Key point
洼田饮水试验 Water Swallow Test (Kubota)	吞咽 Swallowing	1 级 5 秒内一次咽下无呛=正常
反复唾液吞咽试验 RSST	吞咽 Swallowing	30 秒 ≥3 次正常
VFSS 电视透视吞咽造影	吞咽 Swallowing	金标准 gold standard
FEES 纤维内镜吞咽检查	吞咽 Swallowing	内镜直视咽喉
V-VST 容积-粘度测试	吞咽 Swallowing	评估安全性 + 有效性
GRBAS	嗓音 Voice	主观听感 G/R/B/A/S, 0-3
VHI 嗓音障碍指数	嗓音 Voice	患者自评, 分数越高影响越大
纯音测听 / ABR	听力 Hearing	主观测听 / 客观测听

八、重要英文缩写 Important Abbreviations

学科 Discipline:

- **ST:** Speech Therapy 言语治疗
- **SLP / SLT:** Speech-Language Pathology / Therapy 言语病理学 / 言语治疗
- **PT / OT:** Physical / Occupational Therapy 物理治疗 / 作业治疗

吞咽 Dysphagia:

- **VFSS:** Videofluoroscopic Swallow Study 电视透视吞咽造影
- **FEES:** Fiberoptic Endoscopic Evaluation of Swallowing 纤维内镜吞咽检查
- **RSST:** Repetitive Saliva Swallowing Test 反复唾液吞咽试验
- **V-VST:** Volume-Viscosity Swallow Test 容积-粘度吞咽测试
- **FOIS:** Functional Oral Intake Scale 功能性经口摄食量表
- **NMES:** Neuromuscular Electrical Stimulation 神经肌肉电刺激

失语症 Aphasia:

- **PACE:** Promoting Aphasics' Communicative Effectiveness 促进实用交流能力
- **MIT:** Melodic Intonation Therapy 旋律语调治疗
- **AAC:** Augmentative and Alternative Communication 辅助与替代交流

嗓音 Voice:

- **GRBAS:** Grade-Roughness-Breathiness-Asthenia-Strain 嗓音听感评估
- **VHI:** Voice Handicap Index 嗓音障碍指数
- **TEP:** Tracheoesophageal Puncture 气管食管穿刺

听力 Hearing:

- **CAPD:** Central Auditory Processing Disorder 中枢听处理障碍

- **ABR:** Auditory Brainstem Response 听性脑干反应
 - **OAE:** Otoacoustic Emission 耳声发射
-

九、考试重点提示 Exam Focus Points

1. 名解先拿满 **Master term explanations first:** 语言/言语 + 各障碍定义, 能默写英文 write in English。
 2. 失语四维度鉴别 **Aphasia 4-axis differentiation:** 流利性 / 听理解 / 复述 / 命名 → 定型。
 3. 吞咽两组背熟 **Two key dysphagia sets:** 气道保护 4 手法、进食姿势 4 调整。
 4. 嗓音盯 **GRBAS Watch GRBAS:** 五项含义 + 0-3 评分。
 5. 选择记数字与"最常见" **MCQ facts:** 分级 dB、洼田/RSST 数值、用声过度=最常见病因。
 6. 注意中英文专业词汇对照 **Mind the bilingual terms:** 英文卷, 重在能写出英文。
- 评分 **Grading:** 选择 40 + 名解 20 + 简答 20 + 论述 20 = 100, ≥60 及格 passing。